



Addis Ababa University
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Law and Ethics Assignment

Group Members

- FETHIYA ABDULFETARE WABELA
- FEWZAN ABDULKAF MUSSA
- FOYAT FEKADU DEMSSES
- GEMECHIS FEKEDE TAYE
- GETINET AMHA HAILU
- HAFSA MOHAMMED WEGBELA
- HANA TIBEBU MURAGA
- HAWI NIGUSIE HAWAS
- HEAVEN SOLOMON MEKONEN
- HELINA WENDMAGEGNEHU TUJI
- HEWAN GETENET BEGNA
- IMAN REDWAN JEMAL
- KALKIDAN BAHARU TADESSE
- LIYA AMBACHEW NEGUS
- LOZA TEKALIGN GEBRESLASSEI
- MARAKI SULIMAN IBRAHIM

ID Number

UGR/2015/15
UGR/9221/15
UGR/3673/15
UGR/4060/15
UGR/0882/15
UGR/2880/15
UGR/9168/15
UGR/4175/15
UGR/4692/15
UGR/8344/15
UGR/9183/15
UGR/5362/15
UGR/1929/15
UGR/3454/15
UGR/8288/15
UGR/8635/15

Introduction

- Intellectual property (IP) law protects novel drug formulations, manufacturing processes, and brand identities from unauthorized use.
- IP law
 - grants market exclusivity, allowing companies to recoup the high costs of Research & Development for new cures.
 - secures brand identity to guarantee quality and combat dangerous counterfeit drugs.
 - provides a legal framework for transitioning from expensive brand-name drugs to affordable generic competition.

Types of IP & Global Frameworks of IP

- “Patents are central in pharma because developing a medicine can take over a decade and cost huge sums. A 20-year patent gives the company time to earn back that investment.
 - **Patents** (20 years): Exclusive rights that protect new drug molecules, formulations and production methods — they allow developers to recover R&D investments and fund future research.
 - **Trademarks**: Protect brand names and logos (*e.g.*, *Panadol®*), helping pharmacists and patients identify trusted products and preventing misleading substitution.
 - **Trade Secrets**: Confidential manufacturing steps or formulation details kept outside the public domain (no filing required), protected by contract and company policy.
 - **Copyright**: Applies to written materials — research papers, patient leaflets, training manuals, and software used in pharmaceutical operations.
 - **TRIPS (WTO)**: Global baseline for IP rules — sets minimum standards for patents and other IP rights while allowing public health safeguards.

Global Patent Issues & Practical Challenges

- Patent enforcement varies:
 - While patent terms are standard (20 years), how strictly patents are enforced and how quickly they're examined differs among countries.
- Compulsory licensing:
 - Governments can authorize generics for patented drugs during public health emergencies (used in notable HIV/AIDS and COVID-19 cases) to improve access.
- Counterfeit medicines:
 - Weak IP enforcement and porous supply chains lead to fake or substandard medicines — a major patient-safety risk.
- Patent evergreening:
 - Some firms extend market exclusivity by patenting minor changes (new salt forms, formulations) — this can delay affordable generics.
- R&D gap for neglected diseases:
 - Market incentives favor drugs for profitable markets; diseases prevalent in low-income regions often receive limited investment.

Introduction to Intellectual Property Law in Ethiopia

Who formulated it ?

- **Federal Democratic Republic of Ethiopia-**
Enacted the primary law through the legislative process.
- **Ethiopian Intellectual Property Office (EIPO)**
-Drafted, developed, and now administers the law

When was it formulated ?

- Before 2006 – scattered, outdated IP regulations.
- **2006 - Proclamation No. 501/2006**
- Today – modern IP system, creating a unified legal regime for inventions, industrial designs, trademarks, and copyright in Ethiopia

Ethiopia Review Intellectual Law

- Intellectual property rights (including patents) in Ethiopia are governed by **Ethiopian Intellectual Property Office (EIPO)**.
- A patent is granted when an invention is
 - 1) new (novel),
 - 2) involves an inventive (non-obvious) step, and
 - 3) can be applied in industry (industrial applicability).

Cont...

- **Patentable:** pharmaceutical products, chemical compounds, and manufacturing processes.
- **Not patentable:** treatment methods, surgical procedures, and diagnostic methods.
- ❖ Exclusions also cover plant/animal varieties and biological processes.

Cont...

- Standard patent protection lasts **15 years**, extendable by **5 years** if actively used in Ethiopia.
- “Patent of Introduction” allows foreign-patented inventions to be registered locally for up to 10 years.
- Applications must include technical details and claims clear enough for skilled professionals to reproduce the invention.

Global vs. Ethiopia: IP Law in Pharmacy

1. Legal Framework

Global IP System

- Based on WTO TRIPS Agreement
- Uniform minimum standards across member countries
- Strong enforcement of patents and data exclusivity

Ethiopian IP System

- Ethiopia is a WTO observer, not yet fully bound by TRIPS
- IP governed by national proclamations (under modernization)
- Weaker patent enforcement and more public-health flexibility

2. Pharmaceutical Patents

Global Context

- 20-year patent term fully enforced
- Patents cover NCEs, formulations, dosage forms, and new uses
- Patents restrict generic entry until expiry

Ethiopian Context

- Patent protection exists but enforcement is limited
- Many medicines not patented locally, enabling unrestricted generics
- Patent disputes are rare due to limited litigation infrastructure

3. Access to Medicines

Global Context

- Patents often increase drug prices
- Access relies on mechanisms like:
Compulsory licensing
- Developing countries face high financial barriers
- Industry profit focus influences treatment availability

Ethiopian Context

- Priority is affordable essential medicines
- Heavy use of generic imports from India, China
- Government supports local production
- National essential medicines list ensures wider accessibility

4. Regulatory Environment

Global Regulatory Systems

- Strong authorities: FDA, EMA, PMDA
- Patent linkage: generics cannot be approved until patent expires. Strict data exclusivity rules delay generic entry

Ethiopian (EFDA)

- No patent linkage system
- Drug registration focuses on quality, safety, efficacy, not patents
- Generics can enter the market faster

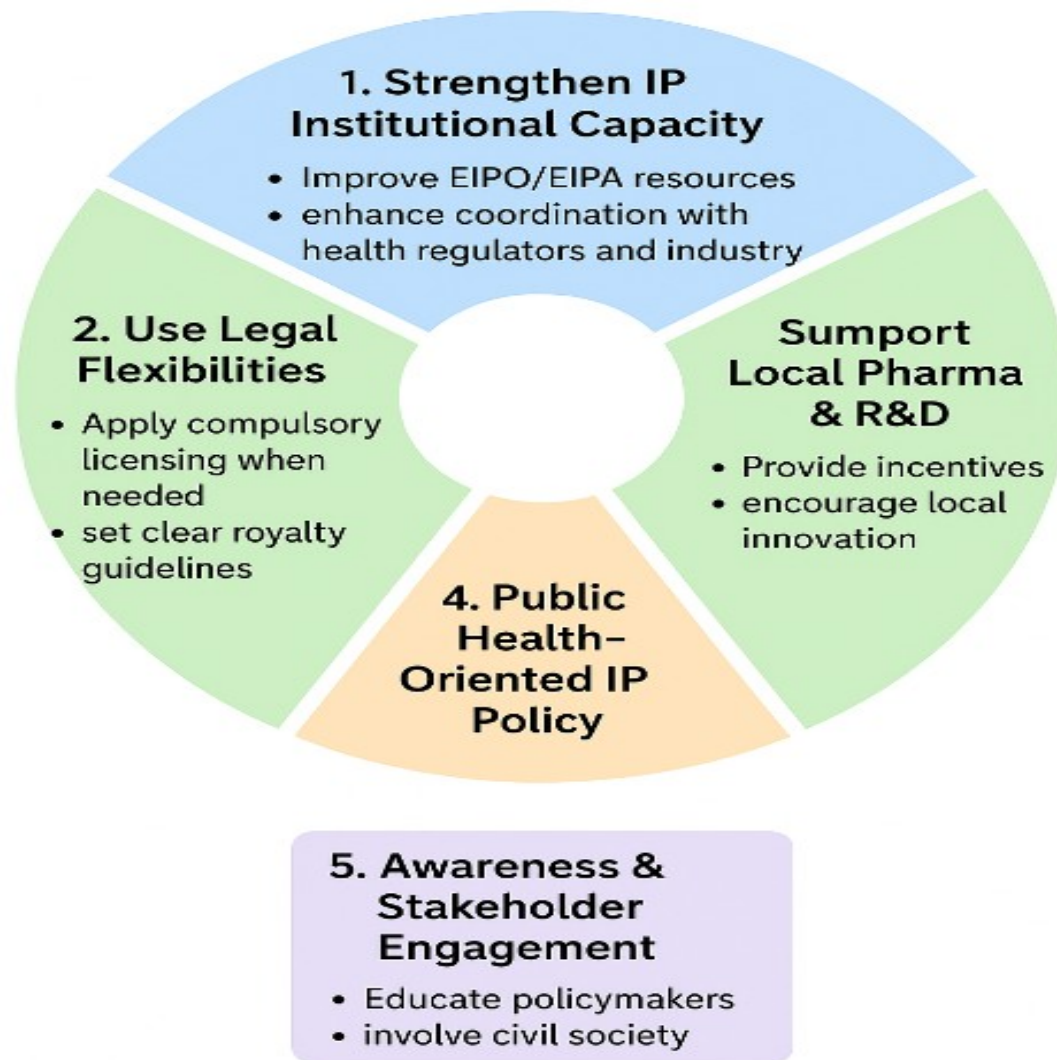
Drawbacks of IP Law in Ethiopia (Pharmacy Context)

- **Weak enforcement:** Limited capacity of EIPO/EIPA leads poor control of counterfeit and unauthorized products.
- **Restricted generic production:** Strong patent protection limits local manufacturers from producing affordable generics.
- **Poor use of TRIPS flexibilities:** Compulsory licensing and government-use provisions rarely applied or difficult to implement.
- **Low R&D and innovation:** Domestic industry produces few generics and invests minimally in research and new drug development.
- **High medicine prices:** Patent-driven monopolies can increase drug costs and reduce access to essential medicines.

Implications of These Drawbacks

- **Limited access to essential medicines:** High prices and restricted generics reduce availability for patients.
- **Stagnant local industry:** Dependence on imports persists due to weak R&D and minimal local production.
- **Public health risks in emergencies:** Poor use of flexibilities delays access to critical medicines during crises.
- **Rise of counterfeit drugs:** Weak regulation increases circulation of unsafe or substandard medicines.
- **Reduced affordability and equity:** High medicine costs worsen financial burden and health inequalities.

Suggestions and Solutions



Reference

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